

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

HOSPICE ELIGIBILITY QUICK REFERENCE

Clinical Criteria by Diagnosis for Field Use

A complete field reference covering eligibility criteria, clinical indicators, LCD documentation terms, and conversation guidance for eight major terminal diagnoses.

Hospice Eligibility Quick Reference

Know the Criteria, Know the Patient, Serve the Family

UNIVERSAL ELIGIBILITY REQUIREMENTS

Regardless of diagnosis, every patient must meet ALL of the following criteria before electing the Medicare Hospice Benefit. Know these by heart.

- Enrolled in Medicare Part A (Hospital Insurance)
- Terminal prognosis of 6 months or less if disease runs its normal course, certified by both the hospice medical director and the attending physician
- Patient or legal representative signs a hospice election statement agreeing to forgo curative treatment for the terminal diagnosis
- Patient resides in the hospice service area

Key clinical note: A patient who lives longer than 6 months does NOT lose eligibility. They are recertified every benefit period (90, 90, then 60-day periods) as long as clinical decline continues. This is one of the most important points to explain to referral sources who delay because they are "not sure it's time."

CONGESTIVE HEART FAILURE

Primary Eligibility Indicators

- NYHA Class IV: Symptoms of cardiac insufficiency at rest
- Ejection fraction at or below 20% despite optimal medical management
- Refractory angina or symptomatic dysrhythmias not controlled by medication
- Three or more inpatient hospitalizations in the preceding 12 months for CHF exacerbation

Supporting Clinical Indicators

- Persistent hypotension (systolic below 90 mmHg)
- Renal insufficiency with creatinine above 3.0 mg/dl
- Cardiac cachexia: significant unintentional weight loss
- Hyponatremia: serum sodium below 130 mEq/L

- Patient declining or not tolerating recommended therapies

Key Documentation Terms for This Diagnosis

- Optimally medically managed, Refractory heart failure, Volume overload despite diuresis
- Goals of care aligned with comfort, Functional decline consistent with terminal trajectory

COPD AND PULMONARY DISEASE

Primary Eligibility Indicators

- Disabling dyspnea at rest or with minimal exertion despite maximal bronchodilator therapy
- FEV1 below 30% of predicted value on post-bronchodilator pulmonary function testing
- Hypoxemia at rest requiring supplemental oxygen (PO2 55 mmHg or less on room air, or SpO2 88% or less)
- Hypercapnia on ABG (PCO2 50 mmHg or more)

Supporting Clinical Indicators

- Cor pulmonale or right heart failure secondary to pulmonary disease
- Unintentional weight loss exceeding 10% body weight over 6 months
- Resting tachycardia above 100 beats per minute
- Progressive functional decline despite therapy and rehabilitation

CANCER

Primary Eligibility Indicators

- Metastatic or locally advanced disease not amenable to further curative treatment
- Patient has declined further curative treatment or is no longer a candidate
- Palliative Performance Scale (PPS) at or below 70%: some functional limitation
- Rapid decline over 4 to 6 weeks documented in clinical record

Supporting Clinical Indicators

- Hypercalcemia of malignancy
- Cachexia: weight loss exceeding 10% in the prior 6 months
- Pleural effusion, ascites, or peripheral edema not responding to treatment
- Uncontrolled pain, nausea, or dyspnea requiring escalating medication

- Serum albumin below 2.5 g/dl indicating poor nutritional status

DEMENTIA AND ALZHEIMER DISEASE

Primary Eligibility Indicators (FAST Stage 7 or Beyond)

- Unable to walk without personal assistance — not just walker-assisted, but entirely dependent
- Unable to dress without full assistance from another person
- Unable to bathe without full assistance from another person
- Urinary and fecal incontinence consistently, not occasionally
- Verbal communication limited to 6 or fewer intelligible words in a given day
- Loss of ability to smile meaningfully in response to stimulation

Required Comorbid Conditions (At Least One in Prior 12 Months)

- Aspiration pneumonia
- Pyelonephritis or other upper urinary tract infection
- Septicemia
- Decubiti Stage 3 or 4 (pressure ulcers)
- Fever that recurred after antibiotic treatment
- Difficulty swallowing food or water leading to dehydration or aspiration risk

RENAL DISEASE

Primary Eligibility Indicators

- Creatinine clearance below 10 ml per minute (15 ml per minute for diabetics)
- Serum creatinine above 8.0 mg per dl (6.0 mg per dl for diabetics)
- Patient declining or discontinuing dialysis
- Uremic encephalopathy: confusion, restlessness, somnolence
- Intractable nausea and vomiting unresponsive to standard treatment
- Significant reduction in urine output with fluid overload

STROKE AND NEUROLOGICAL DISEASE

For CVA and Stroke

- Coma or persistent vegetative state persisting beyond 3 days post-event
- Dysphagia severe enough to prevent adequate oral intake and creating aspiration risk
- Post-stroke dementia meeting FAST Stage 7 or beyond criteria
- Karnofsky Performance Scale at or below 40% indicating severe functional impairment

For ALS (Amyotrophic Lateral Sclerosis)

- Critically impaired breathing capacity: vital capacity below 30% of predicted value
- Dysphagia with documented weight loss
- Life-threatening aspiration or the patient is declining artificial ventilation
- Plus one complication in the prior 12 months: aspiration pneumonia, septicemia, fever

ADULT FAILURE TO THRIVE

- BMI below 22 kg per square meter
- Unintentional weight loss exceeding 10% over the prior 6 months
- Serum albumin below 2.5 g per dl
- Declining functional status with no reversible cause identified
- Patient declining workup or intervention for underlying conditions

LIVER DISEASE

- Prothrombin time more than 5 seconds prolonged over control
- Serum albumin below 2.5 g per dl
- Refractory ascites not controlled with diuretics
- Spontaneous bacterial peritonitis
- Hepatic encephalopathy with altered cognition
- Hepatorenal syndrome with rising creatinine

HOW TO USE THIS REFERENCE

Memorize the top 3 diagnoses in your territory. For the others, keep this card accessible. When a referral source says "I have a patient I am wondering about," your clinical knowledge is your most powerful credibility tool. The more accurately you can speak to eligibility criteria, the more referral sources trust your judgment on clinical appropriateness.